

1. Administrative & Study Identification

Full Study Title: A Study Evaluating the Efficacy and Safety of Multiple Therapies in Cohorts of Participants With Locally Advanced, Unresectable, Stage III Non-Small Cell Lung Cancer (NSCLC) **Official Title:** A Phase I-III, Multicenter Study Evaluating the Efficacy and Safety of Multiple Therapies in Cohorts of Patients Selected According to Biomarker Status, With Locally Advanced, Unresectable, Stage III Non-Small Cell Lung Cancer **Short Title/Acronym:** HORIZON-01 **Protocol Number:** B042777 **NCT Number:** NCT05170204 **Posting ID:** 572246595644810362 **Sponsor Name:** Roche (Company: Roche) **Trial Status:** RECRUITING (Status Description: Currently recruiting participants) **Investigational Product:** Biomarker-directed targeted therapies (alectinib [Trade Name: Alecensa, ATC Code: L01ED03, ALK+], Entrectinib [ROS1+], Pralsetinib [RET fusion+]) and Durvalumab **EMA Information:** Alectinib EMA URL: https://www.ema.europa.eu/en/documents/product-information/alecensa-epar-product-information_en.pdf **Phase of Development:** PHASE3 **Medical Monitor:** Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective & Disease Area **Disease Areas:** Non-Small Cell Lung Cancer (Preferred UMLS Name: Stage III Non-Small Cell Lung Carcinoma) **Primary Objective:** To evaluate the efficacy, specifically Progression-Free Survival (PFS), of multiple targeted therapies compared with durvalumab following chemoradiotherapy (CRT) in patients with biomarker-defined locally advanced, unresectable stage III NSCLC. **Secondary Objectives:** To assess distant metastasis-free survival (DMFS), time to central nervous system (CNS) progression, objective response rate (ORR), duration of response (DoR), overall survival (OS), and safety (adverse events).

2.2 Background & Rationale To address the critical unmet need for targeted consolidation therapies in patients with unresectable stage III NSCLC who harbor specific oncogene driver mutations (e.g., ALK, ROS1, RET). Following concurrent or sequential chemoradiotherapy (cCRT or sCRT), durvalumab is the standard of care; however, patients with oncogene-addicted tumors may derive greater benefit from specific tyrosine kinase inhibitors (TKIs) like alectinib or entrectinib compared to immune checkpoint blockade.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Platform Study)
Control Type: Active Comparator (Durvalumab) **Blinding:** Open-label (with blinded independent central review for endpoints)
Randomization: 1:1 (Investigational targeted therapy vs. durvalumab control)

3.2 Planned Enrollment & Duration Targeted Enrollment: 71 participants **Number of Sites:** Across 200 sites in 11 countries
Estimated Study Start: 2022 **Estimated Primary Completion:** TBD (Event-driven)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Body weight ≥ 30 kg at screening and age ≥ 18 years with an Eastern Cooperative Oncology Group (ECOG) Performance Status of 0, 1, or 2. 2. Histologically or cytologically documented locally advanced, unresectable Stage III NSCLC of either squamous or non-squamous histology. 3. Prior receipt of at least two cycles of platinum-based chemotherapy given concurrently with radiotherapy (cCRT) at a total dose of 60 (+/-10%) Gy; or sequentially (sCRT). 4. No disease progression during or following platinum-based cCRT or sCRT. 5. Willingness and ability to use the electronic device(s) or application(s) for the electronic patient-reported outcome (PRO). 6. Life expectancy ≥ 12 weeks. 7. Confirmed availability of a representative formalin-fixed, paraffin-embedded (FFPE) tumor specimen. 8. Tumor PD-L1 status determined using the VENTANA PD-L1 IHC SP263 assay (preferred) or the Dako PD-L1 IHC 22C3 pharmDx assay. 9. Eligible biomarker status for corresponding cohorts (e.g., ALK fusion positivity for Cohort A1; ROS1 fusion positivity for Cohort A2). Whole-body PET/CT scan for staging must be performed within 42 days (Cohort A2) or 50 days (Cohort A1) of the first dose of CRT.

4.2 Exclusion Criteria 1. Any history of previous NSCLC, prior treatment for NSCLC, or any evidence of Stage IV disease, including distant metastases (e.g., metastatic non-small cell lung cancer). 2. NSCLC known to have a known or likely oncogenic-driver mutation in the EGFR gene. 3. Grade ≥ 2 pneumonitis or any unresolved Grade > 2 toxicity from prior cCRT or sCRT. 4. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or evidence of active pneumonitis on screening chest CT. 5. Any gastrointestinal (GI) disorder that may affect absorption of oral medications, such as malabsorption syndrome or status post-major bowel resection. 6. Positive for active tuberculosis, hepatitis B (HBsAg), or hepatitis C (unless HCV RNA negative). Participants with HIV are excluded if not well-controlled. 7. Treatment with systemic immunostimulatory agents or immunosuppressive medication within specified washout periods.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Treatment regimens are assigned per biomarker cohort (Cohort A1: alectinib [Trade Name: Alecensa, ATC Code: L01ED03]; Cohort A2: entrectinib; etc.) or Durvalumab standard dosing. **Route of Administration:** Oral (TKIs: alectinib, entrectinib, pralsetinib) or Intravenous (Durvalumab). **Duration of Treatment:** Investigational targeted treatment for up to 3 years OR durvalumab for up to 1 year, until disease progression, unacceptable toxicity, or consent withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care therapies for symptom management. **Prohibited:** Treatment with systemic immunostimulatory agents (e.g., interferon, IL-2) or immunosuppressive medications (e.g., systemic corticosteroids, cyclophosphamide) within specified washout periods prior to study treatment. Live, attenuated vaccines are also prohibited.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Up to -42 to -50	Biomarker testing (local tissue or via BX43361 master screening protocol), Informed Consent, PET/CT/MRI
2	Day 0	Randomization, First Dose Administration
3-12	Weeks 2-12	Clinic visits every 2 weeks for the first 3 months, then monthly: Safety Labs, Toxicity Assessment
13-18	Q1-3 Month Follow-up	Tumor response assessments at regular intervals, Q1-3 Month Follow-up for Survival/Recurrence

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-Free Survival (PFS) by RECIST v1.1. **Measurement Method:** Assessed by Blinded Independent Central Review (BICR) using CT/MRI imaging.

7.2 Secondary & Exploratory Endpoints 1. Distant metastasis-free survival (DMFS) and Time to CNS progression. 2. Objective Response Rate (ORR), Duration of Response (DoR), Overall Survival (OS), and Incidence of Adverse Events.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine clinical and laboratory evaluations during monthly visits; patient-reported outcome (PRO) questionnaires via electronic devices. **Serious Adverse Events (SAEs):** Standard reporting criteria for life-threatening events, hospitalizations, or severe toxicities (e.g., pneumonitis). **Data**

Monitoring Committee (DMC): Independent Data Monitoring Committee appointed by Roche to oversee safety and trial conduct.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Biomarker-specific intention-to-treat (ITT) cohorts (stratified by staging IIIA vs IIIB/IIIC, cCRT vs sCRT, and PD-L1 status). **Sample Size Justification:** Targeted enrollment of \$N=71\$ patients to adequately power PFS comparisons between the pooled/individual targeted therapy arms versus the durvalumab control arm. **Handling of Missing Data:** Standard survival methodology including Kaplan-Meier estimates and right-censoring for patients lost to follow-up or event-free at data cutoff.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** On-site and centralized data monitoring; centralized biomarker confirmation (BX43361 master screening). **Audit Trail:** Standard 21 CFR Part 11 compliant Electronic Data Capture (EDC) systems with eCRF sign-offs and data clarification processes.

11. Study Identifiers & Governance

Primary ID: B042777 **Posting ID:** 572246595644810362 **Registry Number:** NCT05170204 **Sponsor/Lead Organization:** Roche (Company: Roche) **Study Title:** A Study Evaluating the Efficacy and Safety of Multiple Therapies in Cohorts of Participants With Locally Advanced, Unresectable, Stage III Non-Small Cell Lung Cancer (NSCLC) **Official Regulatory Title:** A Phase I-III, Multicenter Study Evaluating the Efficacy and Safety of Multiple Therapies in Cohorts of Patients Selected According to Biomarker Status, With Locally Advanced, Unresectable, Stage III Non-Small Cell Lung Cancer

12. Clinical Framework (Oncology Specific)

Primary Condition: Non-Small Cell Lung Cancer **Preferred UMLS Name:** Stage III Non-Small Cell Lung Carcinoma **Other Related/Excluded Condition Names:** Metastatic Non Small Cell Lung Cancer (Preferred Name: Metastatic non-small cell lung cancer) **Diagnosis Threshold:** Pathologically confirmed NSCLC, stage III, post-chemoradiotherapy **Medical Methodology:** Biomarker-directed precision oncology (ALK, ROS1, RET) **Optimization Target:** Consolidation therapy to extend PFS and delay CNS/distant metastasis

13. Study Procedures & Methodology

Management Pathway: Master screening protocol to allocate patients to actionable oncogene cohorts. **Education Protocol (HCP):** Site initiation visits and RECIST v1.1 training for investigators. **Education Protocol (Patient):** Informed consent and regular PRO questionnaire administration via dedicated electronic applications. **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for primary imaging endpoints.

14. Observation & Follow-Up Schedule

Total Duration: Dependent on disease progression; treatment spans 1 to 3 years followed by survival follow-up. **Onsite Visit Cadence:** Every 2 weeks for the first 3 months, then monthly during treatment. **Remote Monitoring Frequency:** Tele-visits or medical record checks permitted every 1 to 3 months during the survival follow-up phase. **Checklist Review Interval:** Regular tumor imaging at protocol-specified intervals.

15. Sign-off & Approval

	Role		Name		Signature		Date			:---		:---		:---		:---	
	Principal Investigator		[Investigator Name]		_____		[DD-MMM-YYYY]			Clinical Operations Lead		[Operations Lead Name]		_____		[DD-MMM-YYYY]	
			Lead Statistician		[Statistician Name]		_____		[DD-MMM-YYYY]								